Problem Statement

Nevada is experiencing a severe primary care physician shortage, ranking 45th nationally with just 219.6 physicians per 100,000 residents, compared to the national average of 271.2.[1] Over two million Nevadans, or 67% of the population, live in federally designated Health Professional Shortage Areas (HPSAs), leaving significant gaps in access to preventive and primary care services.[1] These shortages disproportionately impact low-income and marginalized populations, who already face systemic barriers to healthcare access. As a result, Nevada has some of the highest rates of preventable chronic conditions, including diabetes and hypertension, which contribute to elevated morbidity and mortality in these communities.[2,7]

The lack of adequate primary care has driven many residents to rely on emergency departments and urgent care centers for non-urgent or preventable conditions, further burdening already overcrowded facilities and escalating healthcare costs.[3,4] For instance, preventable ER visits and hospitalizations are estimated to cost the state millions annually, exacerbating economic strain on Nevada's healthcare system. Compounding the crisis is the aging physician workforce; many primary care providers in Nevada are nearing retirement, worsening the current shortage and underscoring the urgency for sustainable solutions.[5,9]

Expanding Nevada's federally qualified health centers (FQHCs) is an evidence-based, cost-effective strategy to address this crisis. FQHCs provide comprehensive, accessible, and affordable primary care, regardless of patients' ability to pay, and have a proven track record of reducing health disparities.

Issue: Advocate for expanding Nevada's federally qualified health centers (FQHCs) to provide comprehensive, accessible, and affordable healthcare services for low-income and underserved populations.

Issue and Organizational Goals:

- Issue Goal(s):
    - **Primary Goal:**
        - **Expand Nevada's Federally Qualified Health Centers (FQHCs):** Secure state and federal funding to expand the number of FQHCs in Nevada by 5 centers by January 2026, which will directly increase access to primary care for low-income and underserved populations.
    - **Smaller Milestones/Goals:**
        - **Gain Legislative Support:** Obtain public endorsements from at least half of the Nevada state legislature, with a focus on health committee members, to pass a funding bill for expanding FQHCs by mid-2025.

        - **Secure Federal Grants:** Successfully apply for and receive federal grant funding for at least two new FQHC locations in Nevada by mid-2025.
        - **Raise Public Awareness:** Achieve at least 20% increase in media coverage across local and state media outlets within the first 6 months of the campaign, ensuring that the issue of primary care shortages and FQHC expansion gains widespread visibility.
- Organizational Goal(s):
    - **Primary Goal:**
        - **Build Coalition Strength and Influence:** Strengthen the coalition advocating for FQHC expansion by increasing membership by 10 new organizations, including local healthcare providers, advocacy groups, and community organizations by Fall 2025.
    - **Smaller Milestones/Goals:**
        - **Increase Social Media Presence:** Grow the organization's social media following by 25% by March 2025 to enhance advocacy efforts and engage a broader audience in the campaign.
        - **Expand Local Engagement:** Organize 5 community-based events, such as town halls and informational sessions, to educate local residents and garner grassroots support for the FQHC expansion by February 2025.
        - **Develop Local Leaders:** Train at least 10 local community leaders and healthcare professionals in advocacy and policy lobbying techniques, enabling them to effectively influence decision-makers by March 2025.

Allies and Opponents

Existing Allies	Reason(s) for support
1. Nevada Public Health Institute	NPHI is committed to improving health outcomes in Nevada and advocates for expanding Federally Qualified Health Centers (FQHCs) as part of their broader strategy to address healthcare disparities. Their extensive research and policy recommendations make them a powerful ally in pushing for increased access to primary care services for underserved communities, which directly aligns with their mission to reduce health inequities.[10]
2. Local hospital systems	Local hospitals, particularly emergency rooms (ERs), are overwhelmed by patients seeking care for preventable conditions due to the lack of primary care options. Expanding FQHCs will reduce the burden on ERs by providing

	patients with more accessible, affordable, and appropriate care options for non-urgent medical needs. This will lead to improved hospital efficiency, better patient outcomes, and reduced operational costs for hospitals, aligning with their interest in improving the local healthcare infrastructure.[11]
3. Marginalized Communities Advocacy Groups	Marginalized communities advocacy groups have a vested interest in addressing the healthcare disparities that disproportionately affect low-income and marginalized populations in Nevada. These communities experience higher rates of chronic illnesses, like diabetes and hypertension, due to insufficient access to primary care. Expanding FQHCs offers a practical solution to these disparities by improving access to preventive and primary care, which aligns with these groups' goals of promoting social justice and healthcare equity.[10, 12]
1. Nevada Association of Nurse Practitioners	The NANP supports expanding the utilization of nurse practitioners (NPs) in healthcare settings to address provider shortages. FQHCs provide a platform for NPs to play an expanded role in delivering care. This aligns with the association's goals of increasing healthcare access while ensuring NPs have opportunities to practice to the full extent of their training, especially in underserved areas.[10]
2. University of Nevada School of Medicine	The University of Nevada School of Medicine can benefit from this expansion by having more opportunities to train future primary care providers in real-world, community-based settings. Expanding FQHCs will provide both clinical experience for medical students and a career pathway for new healthcare professionals. Additionally, the university has a vested interest in addressing healthcare workforce shortages, making this a mutually beneficial partnership.[12]
Potential Allies	**Reason(s) for support**
1. Nevada Chamber of Commerce	The Chamber of Commerce is likely to support FQHC expansion as it offers a long-term solution to reducing healthcare costs across the state. When individuals have access to primary and preventive care, there is a reduction

	in high-cost emergency care and hospitalizations, ultimately lowering healthcare-related absenteeism and improving productivity. This cost-saving benefit aligns with the Chamber's interests in supporting Nevada's business environment and economic growth.
2. Faith-based organizations	Faith-based organizations often see healthcare as a moral and ethical responsibility, especially for underserved populations. Many religious groups actively promote healthcare as a human right and have a history of supporting community health initiatives. FQHCs align with their values of providing compassionate, accessible care for all, regardless of income or background. Their support could also help mobilize their congregational base to advocate for healthcare reform.
3. Insurance companies	Insurance companies benefit from FQHC expansion because these centers focus on preventive care, which can reduce the incidence of high-cost emergency room visits, hospitalizations, and long-term treatments for chronic conditions. By focusing on preventative care and early intervention, insurance companies can lower overall healthcare spending. This is in their financial interest, as it leads to lower claims and better health outcomes for insured individuals.
4. Major employers (e.g., MGM Resorts, Caesars Entertainment, NV Energy, etc.)	Large employers, particularly those with a significant workforce, can see a reduction in absenteeism and productivity losses when their employees have access to primary care. Providing workers with affordable, accessible healthcare through FQHCs can reduce workdays lost due to untreated health issues. Major employers have an interest in ensuring the health of their workforce to maintain productivity and reduce the costs associated with employee illness and absenteeism.
5. Medical equipment suppliers (e.g., Medtronic, GE Healthcare, etc.)	Suppliers of medical equipment stand to benefit from the expansion of FQHCs as these centers will increase the demand for medical supplies and diagnostic equipment. As more FQHCs are established, there will be a direct need for additional medical devices, software, and other healthcare technologies. This creates a potential business opportunity for medical

	equipment suppliers looking to expand their market share in Nevada.
Opponents	**Reason(s) for opposition**
1. Private practice physicians	Many private practice physicians are concerned that the expansion of FQHCs will increase competition for patients, particularly in areas with existing primary care providers. FQHCs may offer lower costs and more accessibility, which could attract patients who would otherwise seek care from private practices. This could result in decreased patient volume and revenue for private providers.
2. Nevada Taxpayers Association	Anti-tax groups generally oppose any increase in federal or state spending, especially for public health initiatives. They may view the expansion of FQHCs as an unnecessary burden on taxpayers, especially if it requires increased state or federal funding. Their primary concern is likely to be the financial cost and potential for higher taxes or reallocated funds.
3. Some conservative legislators	Conservative lawmakers may resist the expansion of FQHCs due to concerns about government overreach and the perception that federal or state funds are being used to expand government-controlled healthcare. They may view this as an unwarranted expansion of federal or state influence in the private healthcare sector and argue against the involvement of government in healthcare delivery.
4. Insurance lobbying groups (e.g., Nevada Association of Health Plans)	Insurance companies may oppose the expansion of FQHCs if they believe it could reduce their market share or profit margins. While FQHCs improve access to care, they may be seen as an alternative to private insurance. These groups could argue that the expansion of publicly funded centers takes away from their ability to control healthcare costs and profit from private health insurance models.
5. Property developers	Property developers may oppose FQHC expansions in certain areas due to

	concerns over zoning, land availability, and property values. If land is allocated for new FQHCs, it could reduce the opportunities for commercial or residential development in those areas. Developers may view the allocation of land for public health facilities as a financial setback, particularly in growing urban or suburban regions.

Arguments and Counterarguments

1. **Arguments**	**Counterarguments**
1. Expanding FQHCs will be too costly for taxpayers.	FQHCs are primarily funded through federal grants, which reduces the financial burden on the state. Additionally, the cost savings from reduced emergency room visits and hospitalizations outweigh the initial investment.[13]
2. There aren't enough doctors to staff new FQHCs.	FQHCs can hire nurse practitioners, physician assistants, and other healthcare professionals, not just doctors, to fill the gap in care.[13]
3. Existing urgent care centers and ERs already provide adequate care for underserved populations.	Urgent care centers and ERs focus on immediate treatment, not preventive care or follow-up, which leads to higher long-term healthcare costs and worse health outcomes.[14]
4. FQHCs won’t solve healthcare disparities; more comprehensive healthcare reform is needed.	While broader reforms are important, FQHCs offer an immediate, effective solution to provide care for underserved populations and address disparities.[14,15]
5. Expanding FQHCs will take too long to have an impact.	Evidence from other states shows that FQHCs can be rapidly expanded with federal support, and health outcomes improve within a few years of implementation.[14,15]

Targets

Name	Title	Decision-making Body	Goal for This Target	Notes
Primary				
Joe Lombardo	Governor of Nevada	Executive Office of Nevada	Secure executive support for state funding and lobbying efforts	Holds executive authority to influence state budgets and policies. Needs evidence of cost-effectiveness and public benefit.
Dina Titus	U.S. Representative, District 1	U.S. House of Representatives	Advocate for increased federal grant funding for FQHCs in Nevada	Serves on committees related to public health. Represents urban areas with significant underserved populations.
Richard Whitley	Director, Nevada DHHS	Nevada Department of Health and Human Services	Approve and allocate state resources for FQHC expansion	Oversee state healthcare initiatives. Can influence state-level healthcare policy implementation and grant applications.
Secondary				
Marilyn Dondero Loop	Chair, Senate Health and Human Services Committee	Nevada State Senate	Support legislative approval for state healthcare funding	Key leader in legislative healthcare decisions. Has supported similar initiatives.
Jason Frierson	U.S. Attorney for Nevada	U.S. Department of Justice	Advocate for public health funding as crime prevention strategy	Represents federal government in Nevada; connects health access to broader social issues.
Mark Amodei	U.S. Representative, District 2	U.S. House of Representatives	Build bipartisan support for FQHC funding at the federal level	Senior congressional member with influence across party lines; represents rural areas that would benefit from FQHCs.

Nicole Cannizzaro	Nevada Senate Majority Leader	Nevada State Senate	Secure caucus support for state-level healthcare investments	Can influence Senate votes on healthcare bills and funding priorities.
Steve Sisolak	Clark County Commissioner	Clark County Board of Commissioners	Allocate county-level resources for FQHC locations in urban areas	Former governor; has insights and networks to mobilize local resources for public health.
Keith E. Whitfield	President, UNLV	UNLV Administration	Collaborate to establish medical training programs for FQHC staff	Leader of a major academic institution; UNLV can help address workforce shortages through education and training programs.

Stakeholders

Name/Organization	Support	Opposed	Support if amended	Who will contact?	Notes
Nevada Public Health Institute (NPHI)	Yes	No	N/A	Coalition Leadership	A key advocate for healthcare equity in Nevada. Can provide data and mobilize grassroots support for the expansion of FQHCs.
Nevada Hospital Association (NHA)	Yes	No	N/A	Coalition outreach team	Supports initiatives that alleviate ER overcrowding. Collaboration could improve resource allocation and care transitions.
Nevada Medical Association (NMA)	No	Yes	Possibly	Policy team	Represents private practice physicians who may feel competition. Could support if FQHCs focus on underserved rural areas.
Faith-Based Organizations (e.g.,	Yes	No	N/A	Local advocacy	Strong advocates for equitable healthcare. Willing to engage congregations and leaders

Catholic Charities of Southern Nevada)				leaders	to mobilize community support.
United Health Group	No	Yes	Yes	Liaison from coalition team	A major insurance provider that might oppose the initiative unless it demonstrates cost savings and operational benefits.
Nevada Chamber of Commerce	No	Yes	Yes	Economic developmen t team	Concerned about funding through increased taxes. Could be swayed if the proposal emphasizes economic benefits and cost savings.

Campaign Strengths and Weaknesses

Strengths	
1. Strong data and research support: The campaign is supported by compelling data on Nevada's healthcare needs, particularly the shortage of primary care providers, and the effectiveness of FQHCs in addressing healthcare access issues. The state's Health Professional Shortage Areas (HPSAs) and the economic cost of emergency room overcrowding provide solid evidence to support the expansion of FQHCs.	NA
2. Broad coalition of allies: The campaign has a wide range of potential allies, including public health organizations (e.g., Nevada Public Health Institute), local hospitals, and underrepresent communities advocacy groups. These organizations bring credibility, expertise, and grassroots support to the initiative.	
3. Cost-effectiveness of FQHC expansion: Expanding FQHCs is a cost-effective solution that addresses both	

healthcare access and economic sustainability. Evidence shows that FQHCs can reduce long-term healthcare costs by preventing expensive ER visits and hospitalizations.	
4. Public and legislative awareness: The issue is timely and resonates with both the public and legislators. Rising healthcare costs, especially in underserved areas, are a priority for voters and lawmakers alike. This creates a strong foundation for mobilizing both local and state-level support.	
5. Public Health impact: The expansion of FQHCs addresses not just primary care but also preventive health, mental health services, and chronic disease management, making it a comprehensive solution. The campaign has the potential to significantly improve health outcomes and equity across the state.	
Weaknesses	**How to address**
1. Opposition from private physicians and healthcare providers	Engage private physicians early in the campaign to emphasize that FQHCs complement, rather than compete with, private practices. Stress the benefits of reducing ER overcrowding and freeing up space for urgent and complex cases. Organize meetings and educational forums to address concerns.
2. Concerns over increased taxes or public spending	Frame the expansion as an investment in long-term savings through reduced emergency room visits and hospitalizations. Use data to show that preventive care delivered at FQHCs will ultimately lower overall healthcare spending for the state. Engage with economic analysts to produce cost-benefit studies.
3. Limited awareness of FQHC benefits among key stakeholders	Launch an aggressive public relations campaign to raise awareness about FQHCs' role in reducing health disparities. Include targeted media outreach, especially in rural and underserved areas. Collaborate with local news outlets to highlight personal stories of individuals who have benefited from FQHCs.

4. Political resistance from conservative legislators	Build bipartisan support by focusing on the economic and health equity aspects of FQHC expansion. Engage local business leaders and employers to support the initiative, emphasizing how FQHCs improve workforce health and productivity. Focus on the public health benefits to garner support from both sides of the political spectrum.
5. Challenges in securing sufficient federal and state funding	Develop a clear funding strategy, including identifying potential sources of state and federal funds, such as Medicaid expansion programs and community health grants. Work closely with advocacy groups to lobby for funding increases, and set up a team to apply for federal grants aimed at expanding healthcare services.

Action Plan

Outcomes	Strategies/Objectives	Tactic	Date	Resources	Who's Responsible
Issue Goal 1: Expand Federally Qualified Health Centers (FQHCs) in Nevada					
Expand FQHCs by 5 centers by 2026	Secure state and federal funding for FQHC expansion	Lobby the Nevada state legislature to allocate funding for 5 new centers	January 2026	Research reports, coalition support	Policy team, NPHI
		Organize meetings with legislators and stakeholders to push for funding	February 2025	Contact lists, coalition members	Outreach team
		Work with federal partners to apply for grants to support new FQHCs	February 2025	Federal grant applications, budget plan	Funding team

Organizational Goal 1: Strengthen the Coalition Supporting FQHC Expansion					
Increase coalition membership by 10 organizations by Fall 2025	Build relationships with key healthcare and advocacy groups	Identify and reach out to new potential partners (e.g., local hospitals, marginalized communities advocacy groups)	February 2025	Contact lists, coalition templates	Outreach team
		Host coalition meetings to engage new partners and foster collaboration	May 2025	Meeting logistics, materials	Coalition leadership
		Collaborate with existing allies to identify and recruit new members	February 2025	Existing coalition support	NPHI, Outreach team
Organizational Goal 2: Increase Public Awareness of the FQHC Expansion Effort					
Achieve 20% increase in media coverage by March 2025.	Launch a targeted media campaign about the benefits of FQHCs	Prepare press releases, op-eds, and video content for distribution to local media	January 2025	Press kits, media contacts, video production	Communications team
		Work with local journalists and influencers to highlight stories of communities benefitting from FQHCs	January 2025	Media partners, community interviews	Media liaison team
		Engage local communities through town halls and social media outreach	March 2025	Social media platforms, event spaces	Outreach team, coalition members
Organizational Goal 3: Build Community Advocacy Capacity					
Train 10 local leaders in advocacy	Provide advocacy training for	Host a series of workshops focused on healthcare policy and	January	Training materials, local	Outreach and training team

by March 2025.	community leaders and healthcare professionals	advocacy	2025	leaders	
		Create advocacy toolkits for participants to use in lobbying efforts	February 2025	Printed toolkits, guides	Training team
		Provide mentorship and support to community leaders during the lobbying process	February 2025	Mentorship program, training resources	Coalition leadership

Media Plan

Outlet Name	Medium (Radio, TV, Print, Social Media)	Notes (optional field for further detail about specific media)	Demographics Reached	Access Strategy	Date	By whom
KTNV (Las Vegas)	TV	Local news station with strong coverage in Southern Nevada	Primarily local residents, including urban areas	Pitch a segment on the impact of primary care shortages and how FQHC expansion will address this. Feature community leaders and medical professionals.	January	Communications team
Reno Gazette-Journal	Print	Largest newspaper in Northern Nevada	Adults, residents in Northern Nevada	Submit an op-ed written by a coalition member or healthcare expert highlighting the benefits of expanding FQHCs and the cost savings for the state.	January	Comms director

Nevada Public Radio (KNPR)	Radio	Covers news, healthcare issues, and state-level topics	Educated, policy influencers, health-focused audience	Pitch a radio interview with a public health expert or coalition leader discussing the need for FQHC expansion and how it impacts local communities.	February	Media liaison
Twitter (Official campaign account)	Social Media	Real-time engagement and updates on campaign progress	Wide audience, including younger demographic, activists, and policymakers	Use Twitter to share real-time updates, press releases, and campaign milestones. Engage with local influencers to amplify the message and build momentum.	January	Media team
The Nevada Independent	Print/ Online	Statewide independent news source with focus on politics and policy	Politically engaged residents, policymakers	Pitch an in-depth article covering the specific policy implications and the data backing FQHC expansion, with input from experts and local advocates.	March	Outreach team
Nevada State Legislative News	Print/ Online	Legislative-focused outlet for public policy news	Policymakers, political community	Pitch an article on the legislative needs and benefits of FQHC expansion, backed by data on healthcare costs, morbidity, and mortality.	March	Outreach team

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